# Blank Scenario Template and Guide

**15 yom tractor collision**

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| Scenario Set Up | **Scenario:**<br>● A 15 y/o male was helping his father with chores on their family farm on a Saturday afternoon. After his father drove the tractor through the farm, he got off, and forgot to turn it off. The tractor started rolling down the hill, eventually hitting the boy and knocking him to the ground. The son's right leg was caught under one of the front tires. The father called 911 and stayed beside his son until EMS arrived.<br>**Equipment:**<br>● Blood pressure cuff<br>● Gloves<br>● Tourniquet<br>**Proctor:**<br>● Proctor #1 is son, Proctor #2 is father<br>● For the son: You should be initially anxious/restless → quickly decompensates → becomes lethargic/ unconscious<br>● For the father: You should be anxious and panicking, as you are extremely worried about your son. You should continue to panic throughout the scenario. The EMTs should get info from you. |
| Dispatch | Respond C3 to a 15 year-old male with significant bleeding who was struck and partially run-over by a tractor on a rural farm. |
| Scene Size Up | ● You arrive and find the father, who is frantically waving you down near a stopped tractor<br>● The scene is safe<br>● The EMTs see a boy lying on his back. He is barely conscious, moaning and reaching towards his arm, which has blood soaking through his shirt.<br>● The father is visibly upset: "My son! He's bleeding!! Is he going to be okay??" |

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| Pertinent Primary Assessment Findings | ● AVPU<br>○ Response to verbal<br>● A&O x 2<br>○ A&O to person and place, not time and event<br>● ABCs<br>○ Airway: patent<br>○ Breathing: 30 breaths/min, rapid and shallow<br>○ Circulation: pale, cool and slightly diaphoretic; 142 bpm, cap refill > 3 seconds<br>● Rapid head-to-toe<br>○ Pertinent findings: open fracture on right arm w/ steady bleeding |
| Pertinent Secondary Assessment Findings | **DCAPBTLS**<br>● Head: small forehead hematoma<br>● Neck: no pertinent findings<br>● Chest: bruising + tenderness over lower ribs<br>● Abdomen: bruising on abdomen w/ rebound tenderness<br>● Pelvis: pelvic instability<br>● Extremities: deformity due to open fracture on R arm. CSM findings: very weak distal pulse on R arm, no sensation, can't wiggle fingers)<br>● Back: no pertinent findings<br>.<br><br>**SAMPLE** (provided by father, patient is deteriorating)<br>● A - peanuts, amoxicillin<br>● M - albuterol inhaler<br>● P - asthma<br>● L - ate lunch 1 hr ago<br>● E - pt was outside helping with farm work and got hit by a tractor after his father forgot to turn it off |
| Vitals | **Vitals Before:**<br>● BP: 94/60<br>● HR: 148<br>● RR: 30<br>● SpO2: 95 %<br>**Vitals After:**<br>● BP: 100/68<br>● HR: 132<br>● RR: 16<br>● SpO2: 96% |

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| Treatments | ● Treat for hemorrhagic + hypovolemic shock<br>○ Control extensive bleeding first applying direct pressure around the bone with gauze<br>○ Bleeding continues → apply a tourniquet 2-3 inches above the bleeding site. This stops the bleeding.<br>○ Immobilize right arm w/ a splint (assess CSM before + after applying)<br>○ Apply a pelvic binder (assess CSM before + after applying)<br>○ Load pt into ambulance w/ scoop stretcher, keep pt supine, administer a BVM at 15 liters/min, monitor vitals + mental status every 5 mins, keep warm, rapid transport |
| Key Points | ● Control bleeding first<br>● Treat for shock + continue to monitor vital signs + mental status<br>● Prioritize rapid transport (spend as little time on scene as possible) |
| Bonus Questions | ● Why do you not use a spine board in a patient w/ pelvic instability?<br>○ You need to log roll/shift a patient onto the spine board, which can cause unstable pelvic bones to move + worsen pt's condition<br>● What's the difference between compensated and decompensated shock?<br>○ Compensated: body can still maintain normal blood pressure w/ compensatory mechanisms (like tachy and vasoconstriction)<br>○ Decompensated: when body's compensatory mechanism fail leading to a drop in bp |